

1. CALL FRAME

Unit / dispatch complaint / response mode
Scene type / other agencies + their role
Delays and why

2. ARRIVAL PICTURE

Where found, position, first impression
Who was present
Scene observations that shaped decisions

3. PATIENT

Age / sex / est. weight
Baseline status if known

4. STORY

Chief complaint in their words
Onset / duration / mechanism
Better-worse / before your arrival
Who gave history + reliability

5. PERTINENT NEGATIVES

What the patient specifically DENIED

6. EXAM HIGHLIGHTS

Only findings that drove decisions
or are not in structured fields

7. NUMBERS

Vitals not on upload / trends
Anything abnormal + your read on why

8. THINKING

Working diagnosis
What else you considered + why less likely
Protocol used (name or number)

9. DOING

Each treatment + why
Anything withheld + why
Patient response

10. MOVING

Transport decision + destination rationale
How moved / position + why
Condition on arrival

11. HANDOFF

Report given to whom
Records, lines, devices, belongings

12. EXCEPTIONS

Anything unusual / refused interventions
Delays / equipment issues
What a reviewer should understand

PHOTO SHORTCUTS: monitor screen, med vials, facility paperwork, glove notes. Snap it, send it, confirm the transcription.

HIPAA: no identifiable information in photos. Redact names, DOBs, MRNs, and faces before uploading.

HOURS LATER? Just say so. You'll get targeted recall questions instead of a blank interview.

RULES: If you can't recall it, say so. Gaps get marked [VERIFY], never filled in. You review and own every word before submission.